

University Cancer Institute - Survivorship Care Plan

PATIENT: Evelyn Richardson (born October 18, 1954) – MRN AML066

DATE PREPARED: April 20, 2024

Dear Ms. Richardson,

You have completed your active treatment for Acute Myeloid Leukemia (AML). This document is a summary of your cancer care and a plan for your future health. It is intended for you and all your healthcare providers to ensure you receive coordinated and comprehensive care moving forward.

1. DIAGNOSIS SUMMARY

- **Diagnosis:** Acute Myeloid Leukemia (AML)
- **Date of Diagnosis:** September 9, 2021
- **Key Features:** Your AML was classified as **Intermediate-Risk** by the 2022 European LeukemiaNet (ELN) criteria. This was based on a normal karyotype but the presence of a **FLT3-ITD mutation**.

2. TREATMENT SUMMARY

Your treatment was aimed at eliminating the leukemia and preventing its return. It consisted of two main phases:

- **Phase 1: Induction Chemotherapy (September 17, 2021 - October 15, 2021)**
 - **Chemotherapy:** You received one cycle of intensive "7+3" chemotherapy.
 - **Cytarabine:** Continuous IV infusion for 7 days.
 - **Daunorubicin:** IV push for 3 days.
 - **Targeted Therapy:** Because of the FLT3 mutation, you also received an oral medication.
 - **Midostaurin (Rydapt):** Taken orally twice daily.
 - **Outcome:** You achieved a Complete Remission.
- **Phase 2: Consolidation Chemotherapy (November 2021 - April 2022)**
 - **Purpose:** To eliminate any remaining microscopic leukemia cells.
 - **Chemotherapy:** You received three cycles of High-Dose Cytarabine (HiDAC). Each cycle involved a 5-day hospital stay.
 - **Targeted Therapy:** You continued to take **Midostaurin (Rydapt)** with each consolidation cycle.
- **End of Active Treatment:** April 2022. You have been in continuous remission for **29 months**.

3. ONGOING SURVEILLANCE & FOLLOW-UP CARE

Provider/Test	Frequency	Purpose
Hematology/Oncology Visit (Dr. Vance)	Every 6 months for the next 3 years, then annually.	Clinical check-up, review of bloodwork, assessment for any long-term side effects.
Complete Blood Count (CBC)	Every 3 months for Year 3, then every 6 months for Years 4-5.	To monitor your bone marrow function and for early signs of relapse.
Bone Marrow Biopsy	No longer routinely required unless there is a change in your blood counts or symptoms.	Surveillance

4. POTENTIAL LONG-TERM & LATE EFFECTS OF TREATMENT

Your treatment was very effective but can have long-term side effects. It is important that you and your primary care physician (PCP) are aware of these.

- **Cardiac Health:** The chemotherapy drug Daunorubicin (an anthracycline) can, in rare cases, weaken the heart muscle.
 - **Recommendation:** You should have an **echocardiogram (heart ultrasound)** every 2-3 years. Please inform any new doctor, especially a cardiologist, that you received this drug. Report any new shortness of breath, chest pain, or leg swelling to your PCP.
- **Neuropathy:** High-dose cytarabine can sometimes cause lingering numbness or tingling in the hands and feet.
 - **Recommendation:** Report any significant changes or worsening of these symptoms.
- **Risk of Secondary Cancers:** Chemotherapy can slightly increase the risk of developing other cancers in the future, including skin cancers and other blood disorders.
 - **Recommendation:** Maintain a healthy lifestyle, avoid smoking, use sun protection, and be diligent with all your routine age-appropriate cancer screenings (mammograms, colonoscopies).

5. HEALTH & WELLNESS RECOMMENDATIONS

- **Vaccinations:** Your immune system has recovered, but you should receive all recommended vaccinations, including the annual flu shot and pneumonia vaccines, as if you were never vaccinated before. Avoid live vaccines.
- **Diet & Exercise:** A balanced diet and regular physical activity are crucial for your long-term health and recovery.
- **Emotional Health:** A cancer diagnosis is a life-changing event. Please do not hesitate to seek support from counselors, support groups, or our psychosocial oncology team if you experience anxiety, depression, or fear of recurrence.

We are so pleased with your excellent progress. Please bring this plan to all future medical appointments.

